جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : نهى محمد عبدالقادر الجيلانى
المحافظة : الجيزه **الحي / المدينة :** الهرم **العنوان :** -----
رقم البطاقة : 27012152102488 **رقم التليفون :** 01127538434 **تاريخ تسجيل الإستمارة :** 14-10-2026
العمر(تاريخ الميلاد) : 15-12-1970 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : ورم خبيث بالرحم
نتائج الأبحاث : (مرفق)
التشخيص النهائي : ورم خبيث بالرحم
خطة العلاج : أورام كيماوي علاج تدعيمي وتلطيفي [6669] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نهى محمد عبدالقادر الجيلاني
National ID: 27012152102488

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

A patient of uterine cancer for Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

[[DOCTYPE:NON_HIO_LETTER]]
Non-HIO Letter

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Date Of Submission: ١٤-١٠-٢٠٢٥

Date Of MDT: 10/13/2026 16:46:04

Medical Report

Patient Name :	نهى محمد عبدالقادر الجيلاني	**Age :**	56 Y
Sex :	○ Male ◉ Female	**Comorbidity :**	HTN
ID No. :	27012152102488	**Telephone :**	01127538434
Family History :	MRN: 13879 FH: breast can in Aunt + Cousin	**Marital Status :**	◉ Married ○ Divorced ○ Single
Menopausal Status :	Post	**Reason of MDT :**	
		MDT name::	GU Cancers

Medical Report

Diagnosis : Cervical SCC

Notes :

Edit

55 years, HTN,

Heamaturia --> PA US: Cervical mass

PA MRI + contrast: Large cervical mass + bilateral adenexal masses + Nodal lesions

PET CT 15/9/2026: Large uterine cervical mass + metabolically active supra+ infra diaphragmatic LN

Biopsy: infiltrating SCC

For CT CAP + MDT

History:

Investigations:

Recommendations: Taxol - CArboplatin

MDT DECISION: Taxol - Carboplatin x 6 cycles (with re-evaluation after 3 cycles)
+ Portacath insertion

Specialty	Surgery	Pathology	Radiology

جمهورية مصر العربية

بطاقة تحقيق الشخصية



نهى
محمد عبدالقادر الجيلانى السعيد عليم
٢ ش ٢٦ مارس من ش اللبينى- نوض جلال
الهرم - الجيزه

٢٧٠ ١٢ ١٥ ٢١ ٠٢٤ ٨٨

LC6438994

National ID – Front
[[DOCTYPE:ID_FRONT]]

٢٧٠١٢١٥٢١٠٢٤٨٨ ٢٠٢٥/٠٤
ربة منزل

أنثى مسلمة

البطاقة سارية حتى ٢٠٣٠/٠٤/٢٢



National ID – Back
[[DOCTYPE:ID-BACK]]

Patient Name:	***Noha Mohammed Abd El-Kader***	***Patient ID:***	***839285***
		Exam Date:	***9/9/2026***

Report

Post contrast MRI examination of the Abdomen and Pelvis revealed:

Technique of Abdomen:-

- Axial T1, T2 and FIESTA WIs (plates 1, 2 and 3)
- Coronal T1 and T2 WIs (plates 4 and 5)
- Post contrast series (plates 6 and 7)

Technique of Pelvis:-

- Coronal T1, T2 and STIR WIs (plates 1, 2 and 3)
- Axial T1 and T2 WIs (plates 4 and 5)
- Sagittal T2 WIs (plate 6)
- Post contrast series (plates 7, 8 and 9)

Findings:-

- The hallmark of the study is the presence of a large globular shaped endophytic uterine cervical soft tissue lesion (about 5.8x8.8 x 8.5 cm in maximum axial and CC diameters respectively) as follows;
  - Seen expanding the cervical canal and extending **caudally** at vagina as well, encroaching onto vaginal fornices.
  - **Superiorly** the lesion extends within uterine body (not reaching the uterine fundus) with ill defined myometrial infiltration and distorted endometrial linning.
  - **Anteriorly** abutting the posterior wall of urinary bladder , being inseparable from it at small area.
  - **Posteriorly** seen intimately related to the recto-sigmoid junction and rectum .
  - The lesion is seen displaying intermediate T1 and non uniform hyperintense T2 /STIR signals with restricted diffusion and rather homogenous contrast enhancement. Minimal rim of fluid signal seen within lower uterine segment anteriorly , likely being peri/intralesional.
- Two well defined oval shaped bilateral adenexal soft tissue lesions seen abutting both lateral uterine walls , having similar MR signal as the forementioned cervical lesion with few small cystic non

DOKKI SCAN دقي سكـــان

Patient Name:	***Noha Mohammed Abd El-Kader***	***Patient ID:***	***839285***
		Exam Date:	***9/9/2026***

enhancing foci, the right one measures about 7x4.5x8.5 cm & left one measures about 5x3.1x6 cm in maximum axial and CC diameters respectively.

- Right renal oval shaped lesion of similar signal as forementioned (intermediate T1, intermediate to high STIR / T2 signals with diffusion restriction) measuring about 2.4x2.2 cm in axial diameters.
- Multiple pathologically enlarged bilateral para-aortic, common, internal and external iliac as well as mesorectal lymph nodes, largest at left para-aortic region measures about 3x3.5 cm.
- Enlarged fatty liver with smooth outline and no definite focal or biliary radicles dilatation.
- Patent homogenously enhancing main portal vein and its central branches.
- Normal MRI appearance of the spleen, pancreas, adrenal glands, left kidney, aorta and IVC.
- Non visualized gall bladder ? surgically removed.
- Minimal pelvic ascites.
- Mottled marrow signal intensity and intact cortical outline of pelvic bones (? sequelae of osteoporosis) with intact surrounding musculature.

Conclusion:

- ***Large cervical soft tissue neoplastic growth with bilateral adenexal / abdomino-pelvic nodal lesions as well as right renal soft tissue lesion , with extensions and relations as described .***
- ***Neoplastic disease process with metastatic deposits versus lymphoma are among the diagnostic possibilities , for further clinical and histopathological correlation.***
- ***Fatty hepatomegaly.***

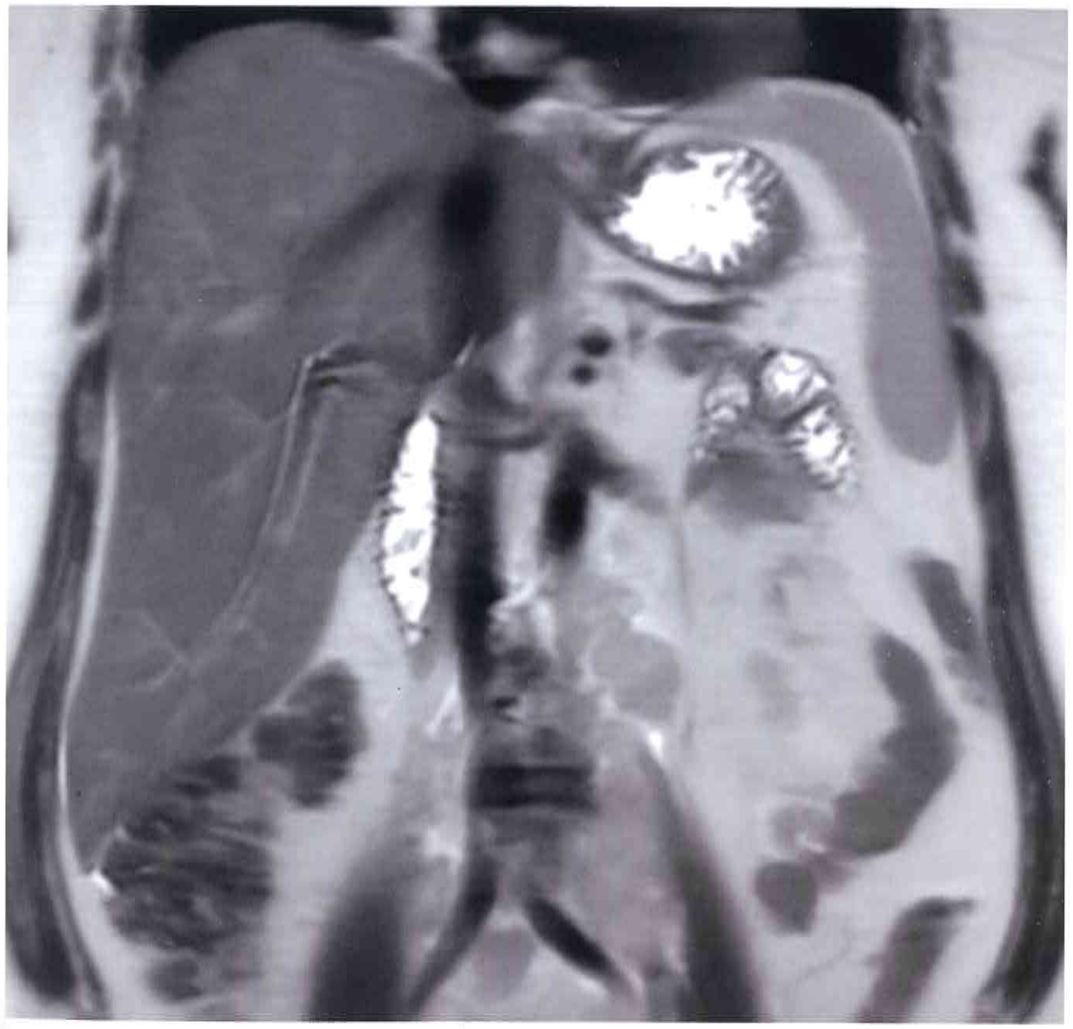
Thanks;

Dr. Shaimaa Khaled, MSc, FEBR ***Dr. Maged Maher, MD***

010 000 101 44 المركز الرئيسي: ٩٢ شارع التحرير - برج ساريدار - الدور الأرضي

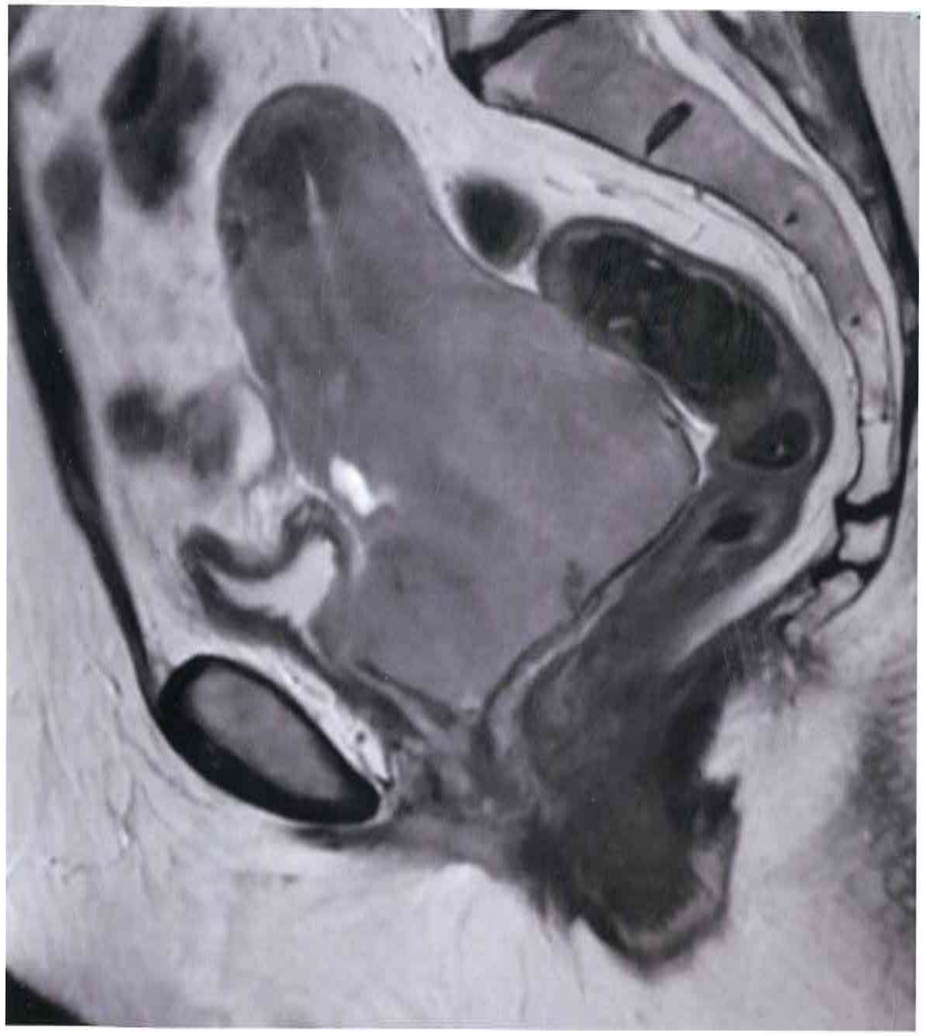
Mammogram [[DOCTYPE:MAMMOGRAM]]

Patient Name:	*Noha Mohammed Abd El-Kader*	*Patient ID:*	*839285*
		Exam Date:	*9/9/2026*



DOKKI SCAN دقي سكـــان

Patient Name:	*Noha Mohammed Abd El-Kader*	*Patient ID:*	*839285*
		Exam Date:	*9/9/2026*

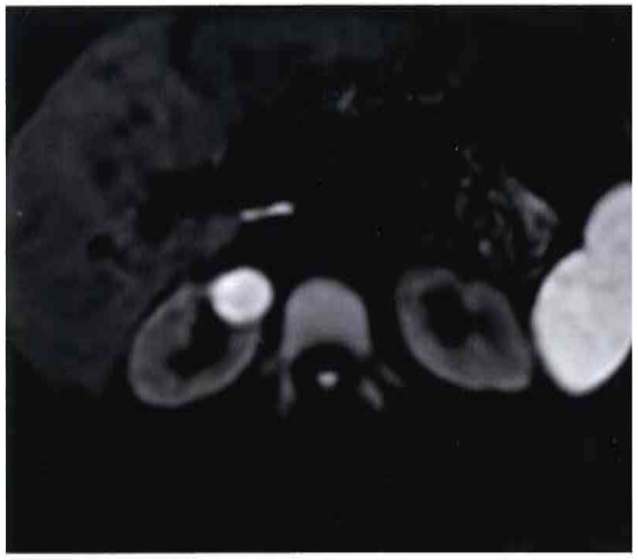


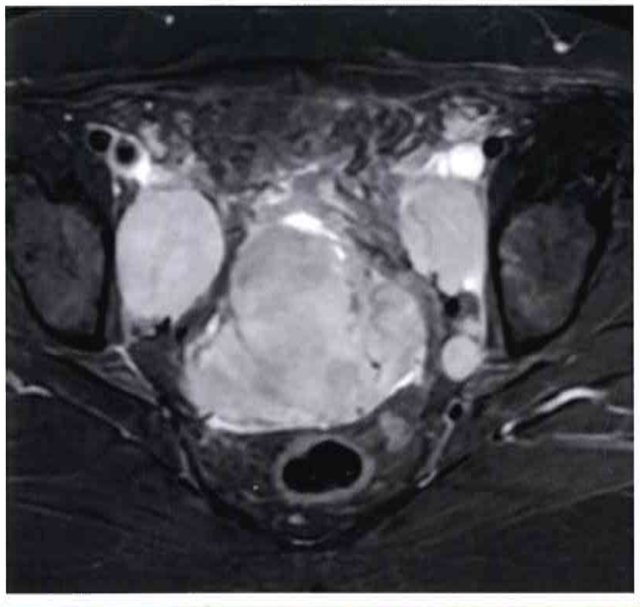
PET Scan [[DOCTYPE:PET_SCAN]]

المركز الرئيسي: ٩٢ شارع التحرير - برج ساريدار - الدور الأرضي 010 000 101 44

DOKKI SCAN دقي سكـــان

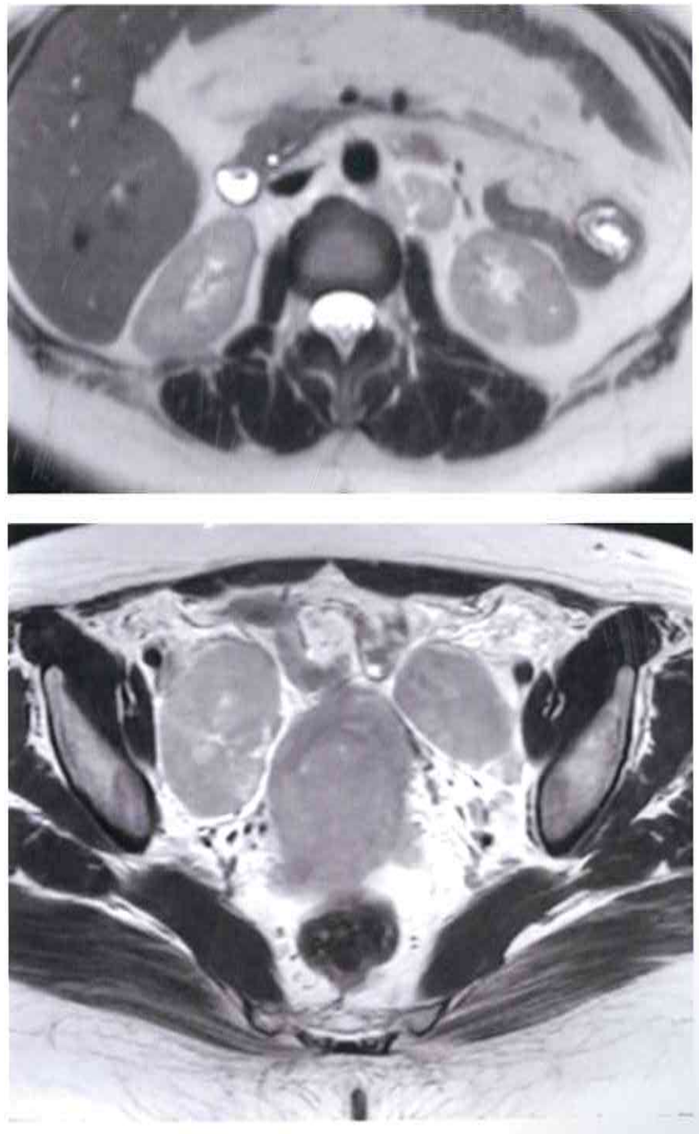
Patient Name:	*Noha Mohammed Abd El-Kader*	*Patient ID:*	*839285*
		Exam Date:	*9/9/2026*





DOKKI SCAN دقي سكـــان

Patient Name:	*Noha Mohammed Abd El-Kader*	*Patient ID:*	*839285*
		Exam Date:	*9/9/2026*



PET Scan [[DOCTYPE:PET_SCAN]]

Whole Body PET/CT Examination

History and Clinical Data:-

Patient data:-

Female patient 55 years old, Weight: 90 Kg, Height: 166 cm, Blood Glucose level: 101 mg/dl

Radiopharmaceutical:-

F-18-FDG 8.2 mCi injected I.V.

Clinical background:-

- A case presented with uterine, adnexal and renal lesions (? lymphoma).

Purpose of the study:-

- Status evaluation.

Technique:-

- Approximately one hour following the IV injection of the radio-pharmaceutical, a total body PET-CT was acquired from the knee to the skull with arms extended above the head. CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- Adequate liver and brain FDG uptake with adequately co-registered PET and CT images.
- Liver showed average SUV ~ 2.02.
- Mediastinal blood pool average SUV ~ 1.23.

Patient Name: *Noha Mohammed Abd El-Kader* *Patient ID:* *839285*
260913345 *Exam Date:* *15/9/2026*

Findings:-

I) *Head and Neck:-*

- Diffuse low grade metabolic activity is noted involving the left thyroid lobe (max SUV ~ 2.57), for further clinical and neck U/S assessment.
- Scanned brain sections show uniform bio-tracer distribution in the cerebral and cerebellar hemispheres.
- No metabolically active (FDG avid) masses are noted.
- Metabolically active few enlarged right lower deep cervical (level IV), right lower posterior triangle and bilateral supraclavicular lymph nodes measuring up to 2.9 cm with max SUV ~ 5.79 (noted at right lower posterior triangle lymph node).



II) *Chest:-*

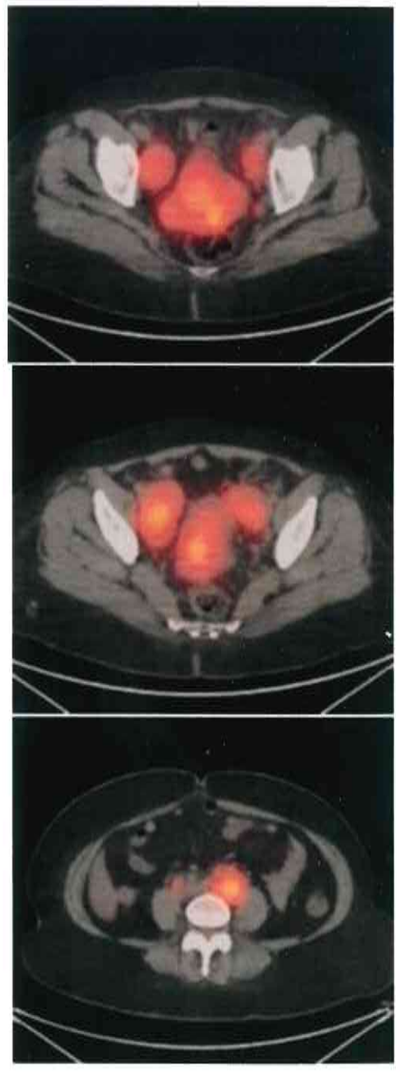
- Metabolically active few enlarged retromanubrial, paratracheal, retrocaval and prevascular lymph nodes are noted measuring up to 1.1 cm with max SUV ~ 2.08 (retrocaval).
- Thin left basal atelectatic band.
- No metabolically active (FDG avid) pulmonary nodules or masses are noted.
- No metabolically active (FDG avid) pleural effusion is noted.
- Normal cardiac size with no pericardial effusion.
- Small hiatus hernia.

PET Scan
[[DOCTYPE:PET_SCAN]]

- Bilateral elastofibroma dorsi (normal variant).

III) ***Abdomen and pelvis:-***

- Metabolically active large lobulated uterine cervix soft tissue lesion with bilateral contour bulge as well as extension to the upper ½ of the vagina and lower uterine body.
- The forenoted lesion is seen measuring 6 x 9.5 x 8.5 cm with max SUV ~ 18.93.
- Metabolically active multiple enlarged paraaortic, aortocaval, portocaval and bilateral iliac (common, internal and external) lymph nodes measuring up to 4.8 x 7 cm with max SUV ~ 17.84 (noted at right external iliac) while the most avid lymph node is noted at left common iliac group measuring 4.1 cm with max SUV ~ 19.66.
- Mildly enlarged liver showing no metabolically active (FDG avid) focal lesions.
- Average sized spleen showing no metabolically active (FDG avid) focal lesions.



- Right renal (medial aspect) isodense lesion is noted measuring 2 cm with no significant increased FDG uptake (likely complicated cyst).

Patient Name: *Noha Mohammed Abd El-Kader* 260913345 *Patient ID:* *839285* *Exam Date:* *15/9/2026*

- The rest of the abdominal and pelvic organs show normal PET/CT appearance with no related focal lesions or abnormal FDG uptake.
- No ascites.

IV) ***Axial and scanned Appendicular Skeleton:-***

- No (FDG avid) metabolically active osteolytic or osteosclerotic lesions are noted.

Conclusion:

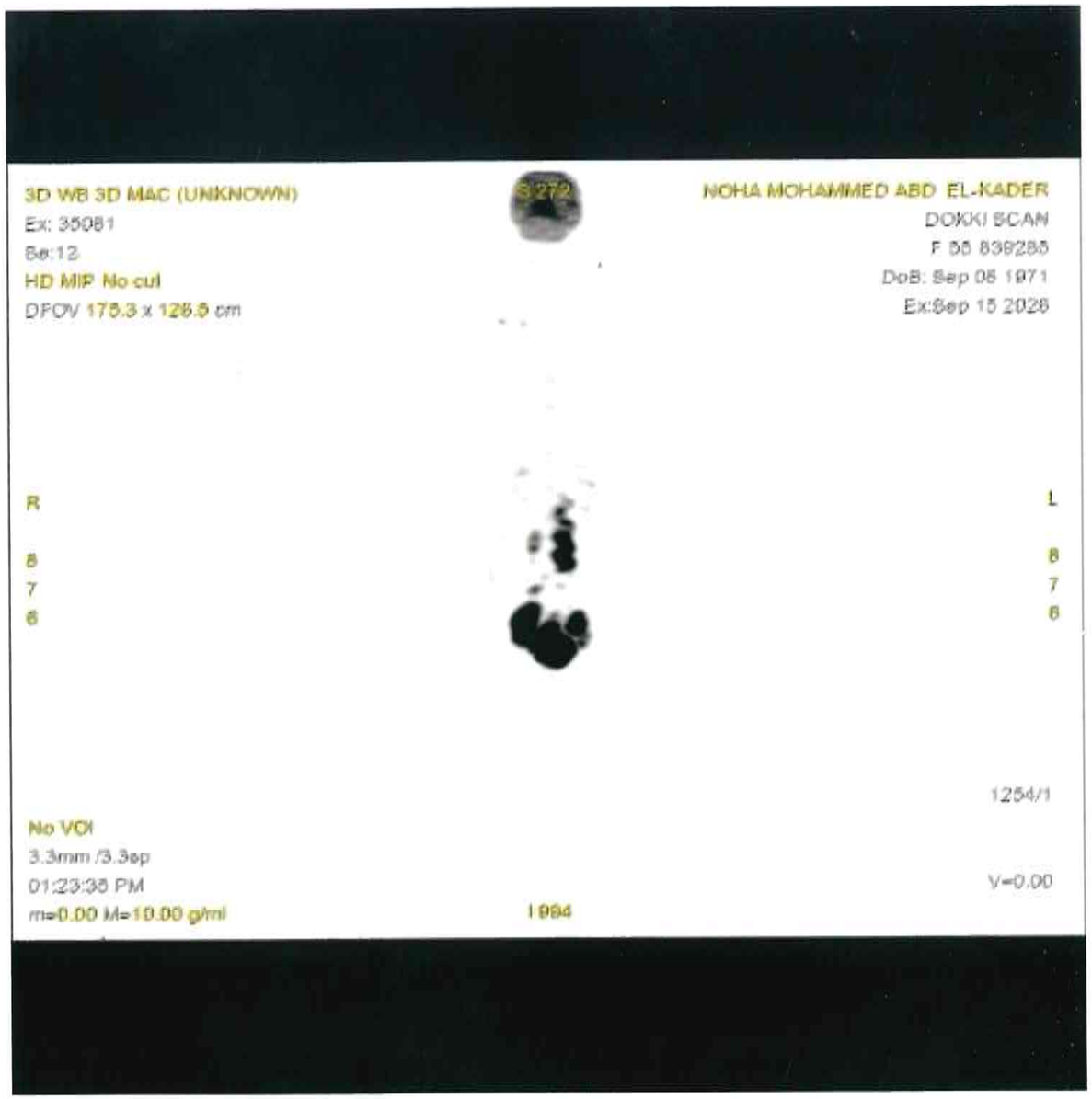
- ***Large metabolically active globular shaped lobulated uterine cervix neoplastic lesion as described.***
- ***Metabolically active multiple supra and infra diaphragmatic nodal lesions.***
- ***Lymphoma is highly suggested, for histopathological correlation.***

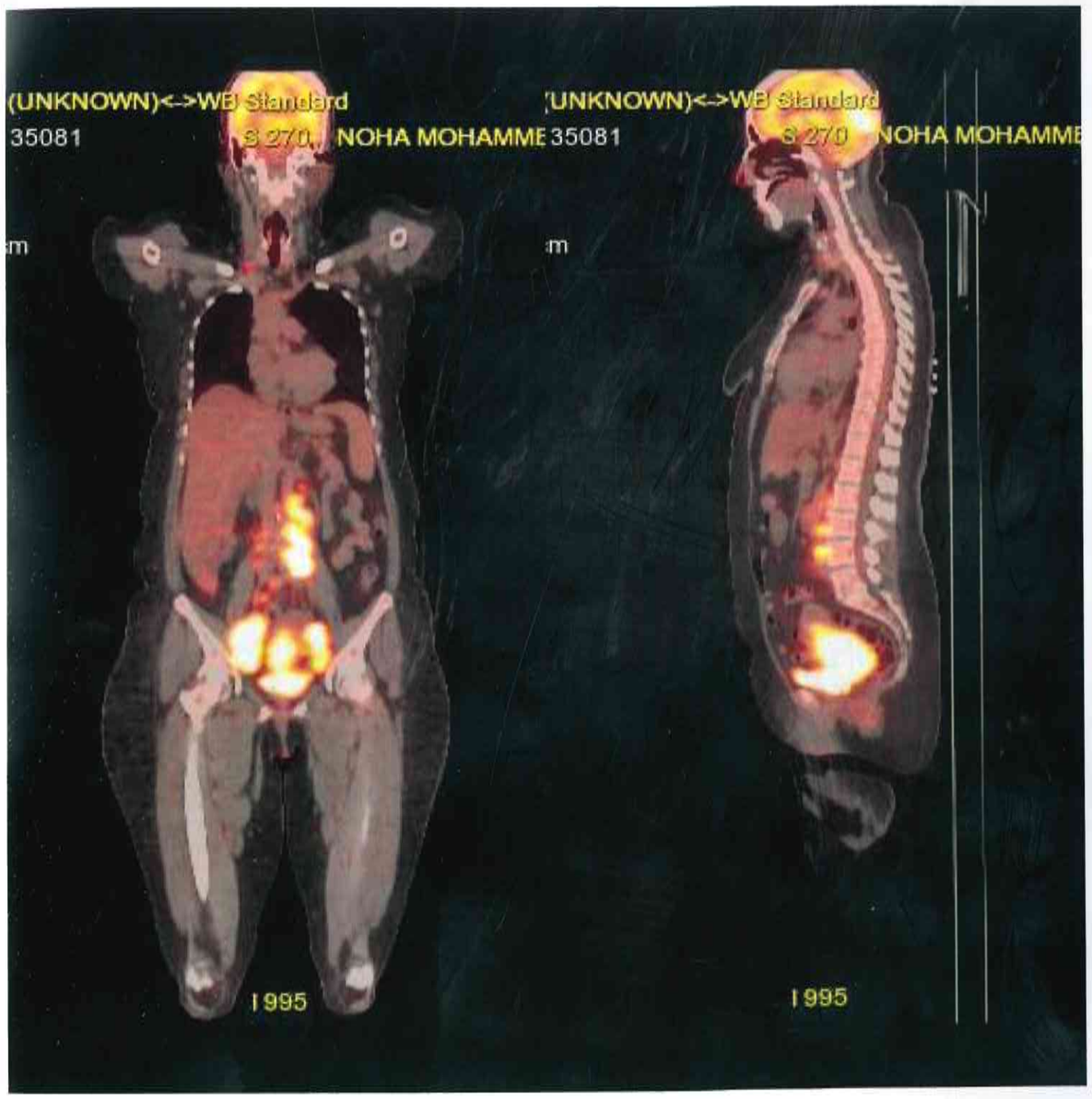
Thank You for Your reference
PET/CT Unit

Dr. Beshoy Nabil, MSc
Revised by Dr / H.A

Radiology consultant
Prof. Dr. Ramy Edward MD

PET Scan [[DOCTYPE:PET_SCAN]]





PET Scan [[DOCTYPE:PET_SCAN]]